KRISHNA MOHAN MEDICAL COLLEGE AND HOSPITAL,
MATHURA

ADVICE FOR BREASTFEEDING

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TITLE:

Advice on breastfeeding includes benefits, correct techniques, counseling tips for post natal mothers regarding advice on breast feeding positions a signs of good attachment.

INTRODUCTION:

Breastfeeding is natural and universally accepted method of infant feeding. Breast milk is the ideal nutrition containing all the nutrients for optimum grow and development of neonate from birth to six months. Exclusive breast feeding for six months is the most effective public health interventions that can reduce neonatal death by 20% and under 5 deaths by 13%.

Exclusive breastfeeding means giving only breast milk. It permits vitamins drops or any medication, if indicated, initiates breastfeeding within 1 hour of birth and continue exclusively for 6 months. After 6 months, start complementary feedings and continue breastfeeding for 2 years as long as the mother wants. Exclusive breastfeeds reduces hospital admissions due to diarrhea and pneumonia by 72% and 57% respectively.

Early initiation of breastfeeding within 1 hour of birth, feeding colostrum as the first immunization colostrum - the yellowish thick milk secreted in just 2-3 days is rich in vitamin A, proteins and immunoglobulins which provide netural immunity to the new borns(1,4). However, NFHS-5 (2019-21), data shows that any 41.8% newborns in india are breastfed within 1 hour and exclusive breastfeeding rate is 63.7%.

The major causes of failure of breast feeding are lack of knowledge regarding correct technique, improper positioning and poor attachment. Incorrect position and attachment leads to sore nipple, cracked nipple, breast engorgement and inadequate milk supply.

For correct breastfeeding, correct positioning and attachment are most important. Four recommended Positions are:-

- Cradle position (most common)
- Cross cradle position
- Football Hold
- Side lying position

A good attachment prevents sore nipples and ensures adequate milk transfers. Four signs of good attachment are:-

- Baby's mouth wide open
- Lower lip everted
- Chin touching the breast
- More areola visible above

AIM:-

To understand the importance of correct advice on breastfeeding and to counsel recently delivered mothers regarding early initiation, exclusive breastfeeding, correct positioning and attachment, and prevention and management of common breastfeeding problems.

OBJECTIVES:-

- To provide practical, structured counseling advice on initiation within 1 hour, exclusive breastfeeding for 6 months, demand feeding and management of common problems for recently delivered mothers.
- To review the current evidence based benefits of breastfeeding for infants and children.
- To demonstrate and explain the correct breastfeeding positions.
- To identify and teach the four key signs of good attachment.

- To summarize determinants, barriers and evidence based interventions to promote exclusive breastfeeding.

METHODOLOGY

Research Design:

A prospective observational study will be conducted to assess breastfeeding advice, knowledge, practices, and awareness among mothers of infants.

Research Setting:

Department of Pediatrics, Krishna Mohan Medical College and Hospital.

Study Population:

Mothers of infants attending the outpatient department, inpatient department, or immunization clinic who are willing to participate in the study.

Inclusion Criteria:

1. Mothers of baby aged 0–24 months.
2. Mothers willing to participate and provide informed consent.
3. Mothers available for an interview regarding breastfeeding advice and practices.

Exclusion Criteria:

1. Mothers unwilling to participate in the study.

2. Infants with galactosemia or admitted to the NICU.

3. Mothers with contraindications to breastfeeding as per applicable clinical guidelines, such as active untreated tuberculosis, certain chemotherapy treatments, or ongoing drug abuse that makes breastfeeding unsafe.

Sampling Technique:

Consecutive sampling of eligible mothers attending the hospital during the study period will be followed.

Variables:

Independent Variables: Maternal age, educational status, occupation, socioeconomic status, parity, mode of delivery, place of delivery, source of breastfeeding advice, and family support.

Dependent Variables: Knowledge regarding breastfeeding, initiation of breastfeeding, exclusive breastfeeding practices, duration of breastfeeding, and awareness of breastfeeding.

Extraneous Variables: Cultural beliefs, family traditions, employment status, maternal health, and access to healthcare services.

Control Variables: Eligibility criteria, a standardized questionnaire, uniform interview procedures, and consistent definitions of breastfeeding practices.

Data Collection:

History, clinical examination, observation of breastfeeding position and attachment, and a structured questionnaire.

Procedure of Data Collection:

1. Eligible mothers will be identified and informed about the purpose of the study.

2. Written informed consent will be obtained before participation.

3. Sociodemographic details and relevant maternal and infant history will be recorded.

4. A structured questionnaire will assess breastfeeding knowledge, sources of advice, initiation, exclusive breastfeeding, feeding frequency, and common breastfeeding difficulties.

5. Information regarding advice received from healthcare professionals and family members will be documented.

6. Data will be checked for completeness and accuracy before analysis.

Ethical Considerations:

Necessary institutional permission and ethical approval, where applicable, will be obtained before commencement of the study. Informed consent will be obtained from all participants. Confidentiality and anonymity will be maintained, and participation will be voluntary.

Time Frame:

The study will be conducted from 5 October, 2026 to 25 October, 2026.

REVIEW OF LITERATURE

Advice for breastfeeding is an important part of neonatal and maternal care and helps in promoting optimal growth and development of newborn. It is useful for improving exclusive breastfeeding rates, preventing neonatal infections and reducing infant mortality. Several guidelines have been developed to support breastfeeding, including the Indian Academy of Pediatrics (IAP) Infant and Young Child Feeding Guidelines, American Academy of Pediatrics (AAP) Policy Statement and standard textbooks like OP Ghai Essential Pediatrics.

Breastfeeding should be initiated within the first hour of life. Colostrum, the thick yellowish milk produced in first few days, is rich in antibodies and should not be discarded(1,4). However in many communities this practice may not be followed because of cultural beliefs, lack of awareness or influence of family members. Early initiation helps in establishment of milk secretion and promotes bonding between mother and baby.

Correct positioning and attachment are key to successful breastfeeding. Four different positions are described - Cradle, Cross-cradle, Football hold and Side-lying position. Good attachment includes four cardinal signs - wide open mouth, lower lip everted outwards, chin touching breast and more areola visible above than below. Incorrect technique leads to sore nipple, breast engorgement and perception of insufficient milk, which are commonest causes of early cessation of breastfeeding(1).

Early initiation within 1 hour, exclusive breastfeeding for first 6 months, avoidance of pre-lacteal feeds like ghutti, honey and water, timely complementary feeding after 6 months and continued breastfeeding up to 2 years(1,2). IAP also emphasizes counseling of mothers on correct positioning, attachment, demand feeding and management of common problems like insufficient milk perception and working mother issues in Indian context.

Exclusive breastfeeding for about 6 months, followed by continued breastfeeding along with complementary foods for 2 years or beyond. AAP also stated that there are very few absolute contraindications to breastfeeding, which include classical galactosemia in infant and maternal conditions like active untreated tuberculosis, chemotherapy and illicit drug use(3,4). The policy also supports expressed breast milk storage and advises against routine bottle feeding.

Various studies have evaluated usefulness and limitations of breastfeeding counseling methods. These interventions are helpful in improving knowledge and practice, but their effectiveness may be affected by maternal education, parity, mode of delivery, working status of mother, family support and experience of counselor. Therefore counseling should be individualized and culturally sensitive.

In conclusion, available literature highlights importance of advice on breastfeeding in routine newborn care. Methods such as antenatal counseling as per IAP, demonstration of position and

attachment as per OP Ghai and support as per WHO Ten Steps and AAP 2022 each have important role.

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